

PRODUCT LITERATURE

TRIOPRIN 300MG TABLET

Each Tablet contains

Aspirin USP 300 mg

Product Description

Round, white coloured tablet.

Pharmacodynamics properties

Mechanisms of action/effect

Salicylates inhibit the activity of the enzyme cyclo-oxygenase to decrease the formation of precursors of prostaglandins and thromboxanes from arachidonic acid.

Although many of the therapeutic effects may result from inhibition of prostaglandin synthesis (and consequent reduction of prostaglandin activity) in various tissues, other actions may also contribute significantly to the therapeutic effects.

Analgesic

Produces analgesia through a peripheral action by blocking pain impulse generation and via a central action, possibly in the hypothalamus.

Anti-inflammatory (Nonsteroidal)

Exact mechanisms have not been determined. Salicylates may act peripherally in inflamed tissue probably by inhibiting the synthesis of prostaglandins and possibly by inhibiting the synthesis and/or actions of other mediators of the inflammatory response.

Antipyretic

May produce antipyresis by acting centrally on the hypothalamic heat-regulating centre to produce peripheral vasodilation resulting in increased cutaneous blood flow, sweating and heat loss.

Pharmacokinetic properties

Absorption and fate

Absorption is generally rapid and complete following oral administration. It is largely hydrolysed in the gastrointestinal tract, liver and blood to salicylate which is further metabolised primarily in the liver.

Indication

For the relief of mild to moderate pain, myalgia, musculo-skeletal pain and other non-rheumatic inflammatory conditions.

Recommended Dose

Analgesic/Anti-pyretic: 1 - 2 tablets

Anti-rheumatic: 2 - 3 tablets

Route of Administration

Oral

Contraindications

Hypersensitivity to aspirin or to any of the excipients listed in section 6.1. Peptic ulceration and those with a history of peptic ulceration; haemophilia, concurrent anticoagulant therapy; children under 16 years and when breast feeding because of possible risk of Reyes Syndrome.

Warnings and Precautions

WARNING

NOT TO BE GIVEN TO CHILDREN UNDER 16 YEARS OF AGE.

AMARAN

TIDAK BOLEH DIBERI KEPADA KANAK-KANAK BERUMUR KURANG DARIPADA 16 TAHUN.

Caution should be exercised in patients with asthma, allergic disease, impairment of hepatic or renal function (avoid if severe) and dehydration. Do not take if you have a stomach ulcer. Do not exceed the stated dose.

If symptoms persist for more than 3 days consult your doctor.

There is a possible association between Aspirin and Reye's syndrome when given to children. Reye's syndrome is a very rare disease which affects the brain and liver, and can be fatal. For this reason aspirin should not be given to children under 16, unless specifically indicated (e.g. Kawasaki's disease).

Interactions with Other Medicament

Other NSAIDs and corticosteroids: concurrent use of aspirin and other NSAIDs or corticosteroids may increase the likelihood of GI side effects.

Diuretics: antagonism of the diuretic effect.

Anticoagulants: increased risk of bleeding due to antiplatelet effect.

Metoclopramide: metoclopramide increases the rate of absorption of aspirin.

However, concurrent use need not be avoided.

Phenytoin: the effect of phenytoin may be enhanced by aspirin.

However, no special precautions are needed.

Valproate: the effect of valproate may be enhanced by aspirin.

Methotrexate: delayed excretion and increased toxicity of methotrexate.

Pregnancy and Lactation

There is clinical and epidemiological evidence of safety of aspirin in pregnancy, but it may prolong labour and contribute to maternal and neonatal bleeding, and so should not be used in late pregnancy.

Aspirin appears in breast milk, and regular high doses may affect neonatal clotting.

Not recommended while breast feeding due to possible risk of Reye's Syndrome as well as neonatal bleeding due to hypoprothrombinaemia.

Side Effects

Side effects are mild and infrequent, but there is a high incidence of gastrointestinal irritation with slight asymptomatic blood loss. Increased bleeding time. Aspirin may precipitate bronchospasm and induce asthma attacks or other hypersensitivity reactions in susceptible individuals. Aspirin may induce gastro-intestinal haemorrhage, occasionally major. It may precipitate gout in susceptible individuals. Possible risk of Reye's syndrome in children under 16 years.

Reporting of suspected adverse reactions

Reporting suspected adverse reactions after authorisation of the medicinal product is important. It allows continued monitoring of the benefit/risk balance of the medicinal product

Symptoms of overdose

Salicylate poisoning is usually associated with plasma concentrations >350 mg/L (2.5mmol/L). Most adult deaths occur in patients whose concentrations exceed 700 mg/L

(5.1 mmol/L). Single doses less than 100 mg/kg are unlikely to cause serious poisoning.

Symptoms

Common features include vomiting, dehydration, tinnitus, vertigo, deafness, and sweating, warm extremities with bounding pulses, increased respiratory rate and hyperventilation. Some degree of acid-base disturbance is present in most cases.

A mixed respiratory alkalosis and metabolic acidosis with normal or high arterial Ph (normal or reduced hydrogen ion concentration) is usual in adults and children over the age of 4 years old. In children aged 4 years or less, a dominant metabolic acidosis with low arterial pH (raised hydrogen ion

concentration) is common. Acidosis may increase salicylate transfer across the blood brain barrier.

Uncommon features include hematemesis, hyperpyrexia, hypoglycaemia, hypokalaemia, thrombocytopenia, increased INR/PTR, intravascular coagulation, renal failure and non-cardiac pulmonary oedema.

Central nervous system features including confusion, disorientation, coma and convulsions are more common in children than adults.

Management

Give activated charcoal if an adult presents within one hour of ingestion of more than 250 mg/kg. The plasma salicylate concentration should be measured, although the severity of poisoning cannot be determined from this alone and the clinical and biochemical features must be taken into account. Elimination is increased by urinary alkalinisation, which is achieved by the administration of 1.26% sodium bicarbonate.

The urine pH should be monitored. Correct metabolic acidosis with intravenous 8.4% sodium bicarbonate (first check serum potassium). Forced diuresis should not be used since it does not enhance salicylate excretion and may cause pulmonary oedema.

Haemodialysis is the treatment of choice for severe poisoning and should be considered in patients with plasma salicylate concentrations >700 mg/L (5.1 mmol/L), or lower concentrations associated with severe clinical or metabolic features. Patients under 10 years or over 70 years have increased risk of salicylate toxicity and may require dialysis at an earlier stage.

Packing

Packed in blister pack of 3 x 10s and 10 x 10s tablets.

Pharmaceutical Precautions

Store below 30°C in a dry place, protected from direct light.

Keep away from reach of children.

Expiry Date

2 years from date of manufacturer

Name and Address of Product Registration Holder/

Distributor:

ZONTRON PHARMACEUTICALS SDN.BHD. (445695-T)

Lot 10 & 11, PERDA Industrial Park,

Lorong IKS Simpang Ampat B,

14100 Simpang Ampat, S.P.S.,

Pulau Pinang, Malaysia

Name and Address of Manufacturer:

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